

An Overview of Abnormal Behavior

Why do some people behave abnormally?

What are the most common kinds of abnormal behavior?

Students in medical school often contract what is known as “medical students’ disease.” Imagine that you are just beginning your training in medicine. One of your textbooks describes “Cryptic Ruminating Umbilicus Disorder”:

“The symptoms are very minor, until the condition becomes hopeless. The first symptom is a pale tongue.” (You go to the mirror. You can’t remember what your tongue is supposed to look like, but it *does* look a little pale.) “Later, a hard spot forms in the neck.” (You feel your neck. “Wait! I never felt *this* before! I think it’s something hard!”) “Just before the arms and legs fall off, there is shortness of breath, increased heart rate, and sweating.” (Already distressed, you *do* have shortness of breath, your heart *is* racing, and you *are* sweating profusely.)

Sooner or later, most medical students decide they have a dreaded illness, usually by misunderstanding the description of some disease and confusing it with their own normal condition. When my brother was in medical school, he diagnosed himself as having a rare, fatal illness, checked himself into a hospital, and wrote out his will. (Today, he is a successful physician.)

“Medical students’ disease” is even more common among students of psychological disorders. As you read this chapter and the next one, you may decide that you are suffering from one of the disorders you read about. Well, perhaps you are, but recognizing a little of yourself in the description of a psychological disorder does not necessarily mean that you have the disorder. All of us feel sad, nervous, or angry occasionally, and many of us have mood swings, bad habits, or beliefs that strike other people as odd. A diagnosis of a psychological disorder should be reserved for people with problems that seriously interfere with their lives.

Concepts of Normal and Abnormal Behavior

How should we define abnormal behavior? If we try to be completely objective, we might define *abnormal* in a statistical manner as any behavior that differs very much from the average. However, by that definition, unusually happy or successful people are abnormal, and severe depression would become normal if enough people suffered from it. Ordinarily, when we say “abnormal,” we mean something other than “different.”

The American Psychiatric Association (1994) has defined



What we consider normal or abnormal depends on the context. Here, people dressed as witches ski down a mountain as part of an annual festival in Basel, Switzerland, in which dressing as witches is supposed to chase away evil spirits. A woman gets painted in stripes in preparation for Mardi Gras celebration in New Orleans. A Malaysian boy kisses a deadly king cobra as part of a competition to win \$2000 for performing various feats with three species of snakes.

abnormal behavior as behavior that leads to distress (pain), disability (impaired functioning), or an increased risk of death, pain, or loss of freedom. We can question that definition as well. For example, when Martin Luther King, Jr., fought for the rights of African-Americans, he risked death, pain, and loss of freedom. But we regard his acts as heroic, not abnormal.

Another way to define abnormal would be to let people decide for themselves whether they are troubled. For example, someone who appears to be successful might say, "Everyone thinks I'm doing fine, but inside I feel miserable." According to a definition that focuses on distress, anyone who thinks he has a psychological problem does have a problem.

Fair enough, but what do we do about people who behave very strangely but insist that they do *not* have a problem? Imagine a woman who babbles incoherently, urinates and defecates in the street, insults strangers, begs for \$1 bills and then sets fire to them, while claiming to be doing all this in obedience to messages from the planet Chlorox. Suppose also that she reports feeling no distress, does not wish to change, and refuses all offers of help. If we call her abnormal or disordered, then what we mean by "abnormal" does not require a feeling of distress. Perhaps, when we say "abnormal," we inevitably imply that the condition is somehow undesirable.

SOMETHING TO THINK ABOUT

How would *you* define abnormal behavior? *

Abnormal Behavior as a Culturally Determined Phenomenon

Each of us learns from our culture the expected or normal behaviors for various situations. We also learn from our culture some of the options for *abnormal* behavior. For example, in one culture in Sudan, women have low status and very limited rights; if a woman's husband mistreats her, she has almost no defense. However, people in this society believe that a woman can sometimes be possessed by a demon, who causes her to lose control and scream all sorts of "crazy" things that she "could not possibly believe," including insults against her own husband (!). Her husband cannot scold or punish her for acting this way, because after all it is not really she, but only the demon who is saying these things. The standard way to remove the demon is to provide the woman with luxurious food, new clothing, an opportunity to spend lots of time with other women instead of with her husband, and almost anything else she demands until the demon departs. You can guess how common demon possession is in this culture (Constantinides, 1977).

More examples: *Brain fog syndrome* is a psychiatric condition with headache, eye fatigue, and inability to concentrate—a common complaint among West African students just before exams. You might try explaining to your own professor that you cannot take a test tomorrow because you

have brain fog syndrome, but unless you live in West Africa, I doubt that your explanation will do you much good. You have probably heard the expression "to run amok." *Running amok* is a type of abnormal behavior recognized in parts of Southeast Asia; someone (usually a young man) runs around engaging in furious, almost indiscriminate, violent behavior (Berry, Poortinga, Segal, & Dasen, 1992). Such behavior is considered an understandable reaction to psychological stress, not a criminal offense.

Does running amok remind you of anything common in North America or Europe? How about the "celebrations" that occur after a sports team wins a major championship? (See Figure 15.1.) Like running amok, we regard such wild displays as temporary, excusable responses to overwhelming emotion.

Although we consider the fans' destructive celebrations to be "normal," their behavior illustrates one important point about conditions that we call "abnormal": Some people who behave "abnormally" are responding to suggestions or examples. For example, one U.S. psychiatrist found that several patients in one mental hospital had cut off one of their ears. Assuming that this behavior must be a common symptom of a psychological disorder, he contacted therapists at other hospitals to ask how often they had observed the same thing. None of them ever had. Apparently, the first patient who cut off his ear gave the idea to the others at the same hospital. (Why they copied that behavior is, of course, an interesting question.)

A related controversy has arisen with regard to **dissociative identity disorder**, previously known as **multiple personality disorder**, in which a person alternates among two or more distinct personalities, each with its own behavioral patterns, memories, and even name, almost as if each personality were really a different person. Such conditions were once considered to be extremely rare. Then,



FIGURE 15.1 Following the victory in a major sports event, the home-town fans often celebrate with a destructive rampage. Under other circumstances, such acts would be considered criminal or insane; under these circumstances, we are more tolerant.



FIGURE 15.2 Chris Sizemore, the real “Eve” in *The Three Faces of Eve*, exhibited a total of 22 personalities, including her final, permanent identity. A person who periodically changes personality and identity is said to have *dissociative identity disorder*. Each of those personalities, by itself, seems reasonably normal. Films, television, and other media often mistakenly refer to this condition as “schizophrenia.” People with schizophrenia have only one personality; that personality, however, is abnormal in serious ways.

beginning in the 1950s, a few such cases received much publicity, such as the case of Chris Costner White Sizemore, featured in the book and movie *The Three Faces of Eve*, written by her psychiatrists (Thigpen & Cleckley, 1957). Sizemore (“Eve”) eventually told her own story, which was very different from her psychiatrists’ version (Sizemore & Pittillo, 1977; Sizemore & Huber, 1988). (See Figure 15.2.) Then a few other people with dissociative identity disorder also received extensive publicity; by the early 1990s, some therapists were reporting many such cases. Why has the apparent prevalence increased so much? According to one hypothesis, patients copy the example of other patients with dissociative identity disorder who they have read about, heard about, or met. Researchers have now assembled convincing evidence against that hypothesis; this is not a condition that people are likely to choose voluntarily (Eich, Macaulay, Loewenstein, & Dohle, 1997; Gleaves, 1996). A more likely hypothesis is that the apparent prevalence of the disorder encourages therapists to look for it and even to suggest to clients—sometimes under hypnosis—that they might have another personality. By doing so, therapists may cause, aggravate, or complicate the problem in vulnerable people (Gleaves, 1996; Spanos, 1994). In short, just as we learn from our culture how we are supposed to act, we also learn—from our culture and even from our therapists—some of our options for abnormal behavior.

The Origins of Abnormal Behavior

Each time and place has interpreted abnormal behavior according to its own worldview. People in the Middle Ages, for example, regarded peculiar behavior as a sign that the disturbed person was possessed by a demon. To

exorcise the demon, priests resorted to prescribed religious rituals (Figure 15.3).

In Western cultures today, the predominant view is the **biopsychosocial model**, which emphasizes that *abnormal behavior has three major aspects—biological, psychological, and sociological*. Many researchers and therapists focus more on one aspect than another, but few would deny that all three are important.

The biological roots of abnormal behavior include genetic factors, which can lead to abnormal brain development, excesses or deficiencies in the activity of various neurotransmitters, hormonal abnormalities, and so forth. Behavior can also be affected by brain damage, infectious diseases, brain tumors, poor nutrition, inadequate sleep, and overuse of certain drugs, including over-the-counter medications.

The psychological component of abnormality consists of a person’s reactions to events. The reactions depend both on the events themselves and on people’s vulnerability. For example, someone with a genetic predisposition or a history of past abuse might be so vulnerable that relatively minor events could trigger a depression. Someone else might become depressed only in the face of a much more serious loss. Regardless of how a problem develops, a psychologist must deal with how a person copes with the current events of life. Depending on the therapist’s theoretical orientation, he or she might try to understand thoughts and motivations or might focus strictly on behavior.



FIGURE 15.3 Peculiar behavior was once explained as demon possession. Here St. Zenobius exorcises devils, fleeing from the mouths of the possessed. At the time of this late 15th-century work, attributed to Botticelli, the priest Savonarola was exorcising the city with public burnings of luxury goods.

Finally, the behavior must be understood in a social and cultural context. People are greatly influenced by how other people act toward them and what other people expect of them. When someone acts strangely, in many cases that person is part of a disordered family or social network, where the others are acting just as strangely, or even worse.

I could say that abnormality depends on both biological and social influences, but that slogan wouldn't tell us much about an individual case. For example, suppose you are a therapist and you meet someone who is crying almost constantly. Before you begin treatment for depression, you need to rule out other possibilities: Maybe the person recently lost a loved one and belongs to a culture that expects people to continue grieving for weeks or months. Another possibility is that the person has a brain tumor that causes uncontrollable crying. Before you can decide how to treat the problem, you need to define the problem as clearly as possible.

The Classification and Prevalence of Psychological Disorders

Any scientific study must be based on agreed standards for classifying information. Imagine what would happen if several psychologists conducted research on the causes or treatment of some disorder (depression, for example) but were all defining depression differently. Chaos would result. Various psychologists might conduct solid research and yet arrive at different conclusions.

That kind of chaos *has* often occurred in psychology, and as a result, psychologists have worked hard to establish uniform definitions and standards for diagnosis. The result of that effort is a reference book called *The Diagnostic and Statistical Manual of Mental Disorders* (DSM)—now in

its fourth edition and therefore known as **DSM-IV**—which lists the acceptable labels for all psychological disorders (alcohol intoxication, exhibitionism, pathological gambling, anorexia nervosa, sleepwalking disorder, stuttering, and hundreds of others), with a description of each disorder.

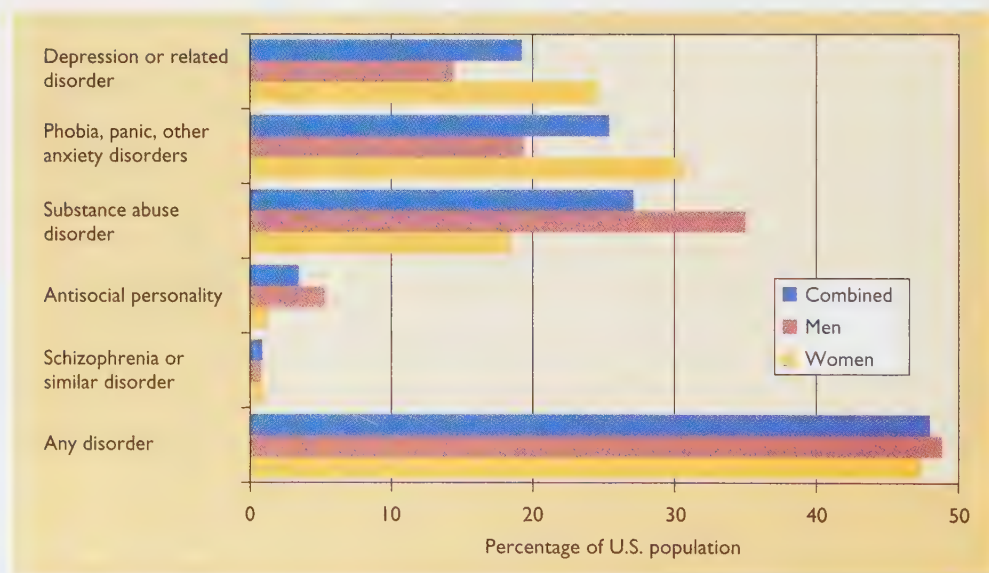
Uses of DSM-IV

DSM-IV functions partly like a nature field guide, describing the defining characteristics of each psychological disorder and how to distinguish it from similar disorders. However, people with psychological disorders do not always fit neatly into one particular category. For example, many have combinations of depression, anxiety, alcohol abuse, and other disorders and may qualify for several diagnoses. Other people have mild problems, requiring a difficult judgment about whether to give them any diagnosis at all.

Psychologists and psychiatrists do their best to make consistent diagnoses, although borderline cases make statistics somewhat uncertain. In one survey of a random sample of about 20,000 people in three U.S. cities, trained interviewers tried to reach all the “usual residents” of each selected neighborhood, including those who lived at home and those who lived in institutions such as prisons, mental hospitals, and nursing homes. They reported that about one fifth of all adults were suffering from a psychological disorder of some sort (as defined by DSM) and that close to one third had suffered from such a disorder at some point (Myers et al., 1984; Robins et al., 1984). A similar study conducted 10 years later estimated that about one half of all people endure a diagnosable psychological disorder at some point in their lives (Kessler et al., 1994). According to both surveys, the most common psychological disorders are phobia and related anxiety disorders, alcohol or drug abuse, and mood disorders (including depression), as shown in Figure 15.4.

Many people are skeptical of any diagnostic system that

FIGURE 15.4 According to one extensive survey, about half of the population of the United States will suffer at least one psychological disorder at some time in life. (The figures for the individual conditions do not add up to the total percentage for “any disorder” because some people have more than one disorder.) However, the exact percentages depend on where one draws the dividing line between “normal” and “abnormal”; for each condition, we can find many borderline cases. (Based on data of Kessler et al., 1994.)



classifies half of all people as psychologically disordered at some point in life. Remember, however, that the exact numbers depend on difficult judgments about how to classify borderline cases. With stricter criteria, the percentages would be lower; with looser criteria, they could be higher.

CONCEPT CHECK

1. Which are the most common psychological disorders? (Check your answer on page 568.)

Criticisms of DSM-IV

Suppose a child who is frequently bullied or ridiculed starts protesting vehemently against going to school, sometimes even vomiting on school mornings. Or imagine a woman who has been in an abusive relationship for years and is now trying to escape from it and restore her damaged self-esteem. Both of these people might consult a psychologist or psychiatrist for help.

In an earlier era, the therapist probably would have talked with them without making any DSM diagnosis. After all, their problems appear to be understandable reactions to their social situations, and most people prefer to avoid the possible stigma of a psychiatric diagnosis. However, today, many insurance policies will pay for visits to a psychotherapist only if the person has a psychiatric diagnosis. Similarly, school systems get extra money to help children with special needs if those children have psychiatric diagnoses. The consequence is that many therapists will try to give almost any client a psychiatric diagnosis. With just a little exaggeration, the child who resists going to school and the woman recovering from an abusive relationship might be said to have one of several anxiety or stress disorders. Similarly, someone who consults a therapist because of diminished sexual pleasure can be diagnosed with *hypoactive sexual desire disorder*; someone who bears a grudge can be diagnosed with *paranoid personality disorder*; and so forth. Of the hundreds of diagnoses described in DSM-IV, many pertain to serious disorders, but some seem to be making normal worries into psychiatric conditions (Kutchins & Kirk, 1997).

DSM-IV's Classifications

The clinicians and researchers who use DSM-IV classify each client along five separate axes (lists). A person may have one or more disorders on a given axis—for example, alcohol abuse and depression—or none at all. The axis that gets the most attention is Axis I, *Clinical disorders*. Table 15.1 lists the major categories of disorder on Axis I. Every disorder on Axis I has its onset at some time after infancy and thus represents a deterioration of functioning. That is, the person is doing worse now than at some point in the past. Axis I conditions include the most common psychological disorders—*anxiety disorders*, *substance abuse*, and *depression*. They also include *schizophrenia*, a condition

that can be extremely disabling. (Chapter 16 will explore anxiety disorders, substance abuse, depression, and schizophrenia in detail.) Axis I also includes eating disorders, sleep disorders, and impulse control disorders—which have all been mentioned in previous chapters—as well as several others.

Axis II, *Personality disorders and mental retardation*, lists disorders that generally persist throughout life (see Table 15.2). These conditions seem to be an integral part of the self, rather than something that a person acquires. A **personality disorder** is a maladaptive, inflexible way of dealing with the environment and other people, such as anti-social behavior or avoidance of other people. Many other psychological disorders resemble medical diseases to some extent, whereas personality disorders are more of an integral part of the person, like being tall, or left-handed, or red-headed. People with personality disorders seldom complain about their condition and seldom seek treatment, except at the insistence of family, acquaintances, or an employer. Even when they do seek treatment, substantial improvement is uncommon.

Axis III, *General medical conditions*, lists physical disorders, such as diabetes or alcoholic cirrhosis of the liver, that can affect a person's behavior. Ordinarily, a psychotherapist does not treat a person for an Axis III disorder but should be aware of the Axis III disorders because they might affect interpretation of behavior.

Stress can intensify a psychological disorder and thus affect the course of treatment. Axis IV, *Psychosocial and environmental problems*, indicates how much stress the person has had to endure, on a scale from 0 (almost no stress) to 6 (stress equivalent to being held hostage or to experiencing the death of a child).

Some people with a psychological disorder are able to proceed with their normal work and social activities; others are not. Axis V, *Global assessment of functioning*, evaluates a person's overall level of functioning, on a scale from 1 (serious attempt at suicide or complete inability to take care of oneself) to 90 (happy, productive, with many interests).

The Importance of Differential Diagnosis

In abnormal psychology, as in any field of medicine, any diagnosis (identification) of a disorder requires ruling out other possibilities. For example, suppose someone says, "I feel unenergetic and pessimistic; I have trouble sleeping at nights; I don't have much appetite; and nothing brings me pleasure anymore." That description sounds very much like depression, but some people have those symptoms as a result of a malfunctioning thyroid gland, a stroke, the side effects from taking certain medications, withdrawal effects after quitting cocaine, a grief reaction following the death of a loved one, or fatigue after a period of being overworked. Similarly, many psychological disorders have symptoms that resemble the effects of various kinds of brain damage,

TABLE 15.1 Some Major Categories of Psychological Disorders According to Axis I of DSM-IV

Disorders usually first evident in childhood or adolescence	Examples: <i>Attention-deficit hyperactivity disorder</i>: impulsivity; hyperactivity; inability to pay attention to school or work <i>Tourette's disorder</i>: repetitive movements, such as blinking an eyelid or twitching a hand; chanted sounds—in some cases, obscene words <i>Elimination disorders</i>: bed-wetting; urinating or defecating in one's clothes <i>Stuttering</i>: frequent repetition or prolongation of sounds, interfering with speech
Substance-related disorders	Alcohol abuse, cocaine abuse, opiate abuse, and abuse of other mind-altering substances
Schizophrenia	Deterioration of everyday functioning, along with either a lack of emotional response, or thought disorders, or hallucinations or delusions
Delusional (paranoid) disorder	Irrational beliefs, such as the belief that “everyone is talking about me behind my back” or that “I have discovered the secret that will solve all the world's problems if I can just get people to listen to me”
Mood disorders	Periods of depression serious enough to interfere with daily life, sometimes alternating with periods of mania, which is the opposite of depression
Anxiety disorders	Lingering anxiety at almost all times, unpredictable attacks of severe anxiety, or periods of anxiety that regularly occur when the person is confronted with a particular object or thought
Somatoform disorders	Examples: <i>Conversion disorder</i>: One or more physical ailments, such as blindness or paralysis, caused at least in part by psychological factors but not intentionally produced <i>Hypochondriasis</i>: Repetitive, exaggerated complaints of illness <i>Somatization disorder</i>: Recurrent complaints of pain and other ailments that are not apparently due to any physical disorder
Dissociative disorders	Loss of the memory of a person's own identity or the memory of past events, not caused by brain damage
Sexual disorders	Examples: <i>Pedophilia</i>: Sexual attraction to children <i>Exhibitionism</i>: Sexual pleasure from exposing oneself in public <i>Voyeurism</i>: Sexual arousal primarily from watching other people undressing or engaging in sexual intercourse <i>Fetishism</i>: Sexual arousal primarily from leather or other inanimate objects
Eating disorders	Examples: <i>Anorexia nervosa</i>: Refusal to eat, fear of being fat <i>Bulimia nervosa</i>: Binge eating alternating with weight loss
Sleep disorders:	Examples: <i>Insomnia</i>: Frequent feeling of not being rested after a night's sleep <i>Sleep terror disorder</i>: Repeated periods of awakening suddenly in an experience of panic <i>Sleepwalking disorder</i>: Repeated episodes of leaving the bed, walking about, and not remembering the episode later
Impulse control disorders	A tendency to act on impulses that other people usually inhibit, such as the urge to gamble large amounts of money foolishly, the urge to steal something, or the urge to strike someone

nutritional deficiencies, hormonal disorders, and so forth. Psychiatrists and clinical psychologists must learn to make a **differential diagnosis**—that is, *a determination of what problem a person has, in contrast to all the other possible problems that might produce similar symptoms*. Only after the therapist knows exactly what the problem is can he or she decide on the best treatment.

THE MESSAGE

Is Anybody Normal?

Most people believe there is an unpleasant stigma attached to being diagnosed with a psychological disorder. Almost any synonym for “mental illness” is considered an insult.

And yet, as you read earlier in this module, according to one study, about one-half of all people in the United States will have at least one DSM-IV disorder at some point in life. If that statistic is accurate, or even close, the implication is obvious: Most of the people who qualify for a psychological diagnosis are not very different from normal. At some point in your life, you may have a bout of depression or anxiety or suffer from substance abuse or other psychological distress. If so, remember that you have plenty of company.

SUMMARY

★ *Normal and abnormal behavior.* Although psychologists and psychiatrists try to be objective and scientific when identifying abnormal behavior, some judgments are necessarily difficult and dependent on value judgments. To

TABLE 15.2 Some Major Categories of Psychological Disorders, According to Axis II of DSM-IV

Mental retardation	Intellectual functioning significantly below average; significant deficits in adaptive behavior
Personality disorders	<i>Examples:</i> <i>Paranoid personality disorder:</i> Suspiciousness; habitual interpretation of other people's acts as threatening <i>Schizotypal personality disorder:</i> Poor relationships with other people; odd thinking; neglect of normal grooming. This disorder is similar to schizophrenia but less extreme <i>Antisocial personality disorder:</i> Lack of affection for other people; tendency to manipulate other people without feeling guilty; high probability of getting into trouble with the law; low probability of keeping a job <i>Borderline personality disorder:</i> Lack of a stable self-image; trouble establishing lasting relationships with other people or making lasting decisions about values, career choice, even sexual orientation; repeated self-endangering behavior, such as drug abuse, reckless driving, casual sex, binge eating, shoplifting, and running up large debts <i>Histrionic personality disorder:</i> Excessive emotionality and attention-seeking; constant demand for praise <i>Narcissistic personality disorder:</i> Exaggerated opinion of one's own importance and a lack of regard for others (Narcissus was a figure in Greek mythology who fell in love with himself) <i>Avoidant personality disorder:</i> Avoidance of social contact; lack of friends <i>Dependent personality disorder:</i> Preference for letting other people make decisions; lack of initiative and self-confidence

avoid tyrannical subjugation of eccentric people, we should be cautious about applying psychiatric labels. (page 561)

★ **Cultural influences on abnormal behavior.** Any culture provides not only examples of how to behave normally but also how to behave abnormally. (page 562)

★ **Multiple causes of abnormal behavior.** Abnormal behavior is the result of various combinations of biological factors, early experiences, and learned responses to a stressful or unsupportive environment. (page 563)

★ **The Diagnostic and Statistical Manual.** Psychological disorders are classified in the *Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition* (DSM-IV). This manual classifies disorders along five axes. Axis I and Axis II deal with psychological disorders; Axis III deals with physi-

cal ailments that can affect behavior; Axes IV and V provide the means of evaluating a person's stress level and overall functioning. (page 564)

★ **Axis I—disorders that affect part of a person's life.** Axis I of DSM-IV lists disorders that usually begin after infancy and that have at least some likelihood of recovery. Three common disorders of this sort are anxiety disorders, substance abuse, and depression. (page 565)

★ **Axis II—lifelong disorders.** Axis II of DSM-IV lists conditions that arise early and persist throughout a lifetime, such as mental retardation and personality disorders. (page 565)

★ **Personality disorders.** Personality disorders are stable characteristics that impair a person's effectiveness or ability to get along with others. Examples of personality disorders are excessive dependence on others and excessive self-centeredness. (page 565)

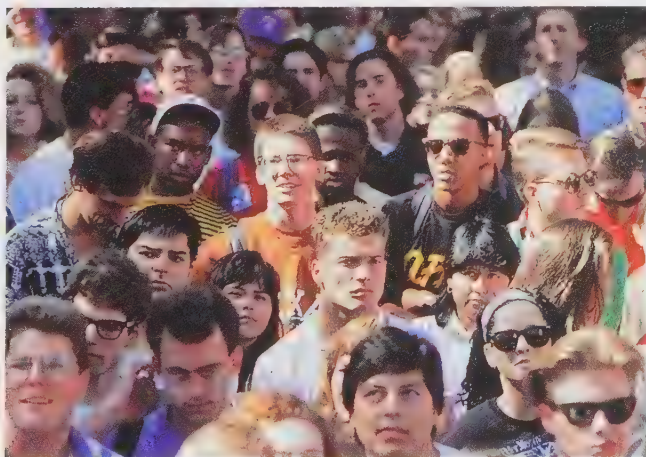
★ **Differential diagnosis.** Psychiatrists and clinical psychologists need to learn to consider all the possible diagnoses of a given set of symptoms and to identify the correct one by eliminating all the other possibilities. (page 565)

Suggestions for Further Reading

- Barlow, D. H., & Durand, V. M. (1995). *Abnormal psychology*. Pacific Grove, CA: Brooks/Cole. A good general textbook on abnormal behavior.
- Holmes, D. S. (1997). *Abnormal psychology*. White Plains, NY: Longman. Another outstanding textbook.

Terms

dissociative identity disorder a rare condition in which the personality separates into several identities; also known as multiple personality disorder (page 562)



Almost everyone has an unpleasant mood or shows strange behaviors once in a while. Many people who qualify for a DSM-IV diagnosis are not much different from anyone else.

biopsychosocial model the concept that abnormal behavior has three major aspects—biological, psychological, and sociological (page 563)

Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition (DSM-IV) a book that lists the acceptable labels for all psychological disorders, with a description of each and guidelines on how to distinguish it from similar disorders (page 564)

personality disorder a maladaptive, inflexible way of dealing with the environment and other people (page 565)

differential diagnosis a determination of what problem a person has, in contrast to all the other possible problems that might produce similar symptoms (page 566)

Answer to Concept Check

1. The most common diagnoses are anxiety disorders, substance-related disorders, and depression. (page 565)

Web Resources

Mental Illness (Overview)

www.psych.org/public_info/overview.html

The American Psychiatric Association provides an overview of Mental Illness in this pamphlet from the *Let's Talk Facts* series.

Psychotherapy

What methods are used to help people overcome psychological disorders?

How effective are these methods?

Observation

If I don't drive around the park,
I'm pretty sure to make my mark.
If I'm in bed each night by ten,
I may get back my looks again.
If I abstain from fun and such,
I'll probably amount to much.
But I shall stay the way I am,
Because I do not give a damn.

—DOROTHY PARKER (1944)

Psychotherapy is a treatment of psychological disorders by methods that include a personal relationship between a trained therapist and a client. But psychotherapy does little good unless the client gives the proverbial damn.



A psychotherapist, like this military psychologist in a Haitian refugee camp, tries to help people overcome problems, or, better yet, to help themselves.

Psychotherapy is used for many well-defined disorders that are listed in DSM-IV and also for adjustment and coping problems. Some psychotherapy clients are virtually incapacitated by their problems, but others are reasonably happy, successful people who would like to function even more successfully.

Before World War II, almost all psychotherapists were psychiatrists, and most of them used Freudian methods. Today, psychotherapy is available from clinical psychologists, social workers, counseling psychologists, and others, using hundreds of methods and approaches that differ markedly. For example, in psychoanalysis and cognitive therapy, the therapist does much of the talking; in person-centered therapy, the therapist says little. Psychoanalysts examine past emotions; cognitive therapists emphasize current emotions; behavior therapists try to change behaviors and practically ignore emotions. Many therapists focus on the problems of the individual; family therapists and community psychologists focus on problems of larger groups. Nevertheless, as you will see, the various forms of therapy have a few fundamental points in common, such as the need for a close relationship between the client and the therapist.

Psychoanalysis

Several types of psychotherapy, known as **psychodynamic therapies**, attempt to uncover people's underlying drives and motivations. For example, both Sigmund Freud's procedure (looking for underlying sexual motives) and Alfred Adler's procedure (looking for underlying power and superiority motives) are considered psychodynamic, despite the differences between them. Here, we shall focus on the procedure developed by Freud, although its practitioners have modified and developed it further since Freud's time.

Psychoanalysis, the first of the "talk" therapies, is a method based on identifying unconscious thoughts and emotions and bringing them to consciousness, to help people understand their thoughts and actions. Psychoanalysis is therefore described as an "insight-oriented therapy," in contrast to therapies that focus on changing thoughts and behaviors (Figure 15.5).

Freud believed that psychological problems were the result of unconscious thought processes and that the only way to control self-defeating behavior is to make those processes conscious. Bringing them to consciousness, he thought, would produce **catharsis**, a release of pent-up emotions associated with unconscious thoughts and memories. Among his methods of bringing unconscious material to consciousness were free association, dream analysis, and transference.

Free Association

Free association is a method that Freud and his patients developed together. (Actually, a more accurate translation of the original German expression would be "free intrusion.")

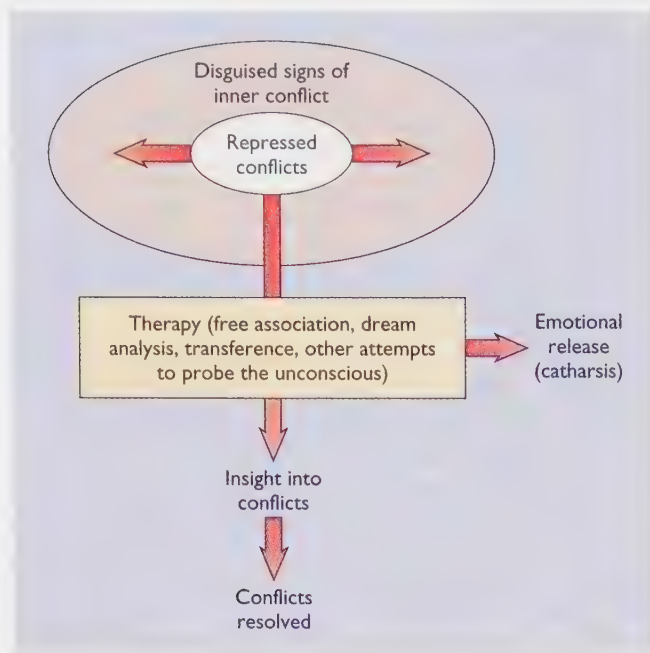


FIGURE 15.5 The goal of psychoanalysis is to resolve psychological problems by bringing to consciousness the unconscious thought processes that are responsible for the difficulty. Analysis literally means “to loosen or break up, to look at the parts.”

In **free association**, the client *starts thinking about a particular symptom or problem and then reports everything that comes to mind—a word, a phrase, a visual image*. The client is instructed not to omit or censor anything or even try to speak in complete sentences.

The psychoanalyst listens for links and themes that might tie the patient’s fragmentary remarks together. The assumption is that all behavior is determined, that nothing happens without a cause. (Recall from Chapter 6 that behaviorists make the same assumption.) Every jump from one thought to another reveals a relationship.

Here is a paraphrased excerpt from a free-association session:

A man begins by describing a conference he had with his boss the previous day. He did not like the boss’s policy, but he was in no position to contradict the boss. He had had a dream. It was something about an ironing board, but that was all he remembered of the dream. He comments that his wife has been complaining about the way their maid irons. He thinks his wife is being unfair; he hopes she does not fire the maid. He complains that his boss did not give him credit for some work he did recently. He recalls a childhood episode: He jumped off a cupboard and bounced off his mother’s behind while she was leaning over to do some ironing. She told his father, who gave him a spanking. His father never let him explain; he was always too strict. (Munroe, 1955, p. 39)

To a psychoanalyst, the links in this story suggest that the man is associating his wife with his mother. His wife was unfair to the maid about the ironing, just as his mother had

been unfair to him. Moreover, his boss is like his father, never giving him a chance to explain his errors and never giving him credit for what he did well.

Dream Analysis

Each dream has a **manifest content**—*the content that appears on the surface*—and a **latent content**—*the hidden content that is represented only symbolically in the actual experience*. Freud (1900/1955) sought to understand the latent content but rejected the view that each item in the manifest content represents the same latent content for everyone. To understand a dream, he said, one must determine what each detail of the manifest content means to the dreamer.

To illustrate, Freud (1900/1955) interpreted one of his own dreams, in which one of his friends was his uncle. He described the following associations: Two of his friends had been recommended for an appointment as a university professor, but both had been turned down, probably because they were Jews. Freud himself had recently been recommended for the same appointment, but he feared that he, too, would be turned down because he was Jewish. Freud’s only uncle had once been convicted of illegal business dealings and had not been very intelligent.

Freud considered one of his friends to be somewhat unintelligent, like the uncle. Another friend had once been taken to court by a woman who accused him of sexual misconduct, although the charges were dropped. By linking these two friends to his uncle, Freud interpreted the dream as meaning, “Maybe they didn’t get rejected for the university appointment because they were Jews, but because one was a simpleton (like my uncle) and the other was accused of a crime (like my uncle). If so, I still have a chance for the appointment.”

To Freud, every dream represents a form of wish fulfillment. The wish may be disguised, but it is always there. For example, in his dream, Freud was not wishing that his friend were his uncle. Rather, he was wishing that he would get the university appointment, and he was wishing that his friends had been rejected for another reason besides being Jewish.

Contrary to what you may have heard, Freud did not see sexual symbolism in all dreams. Curiously, however, the examples he gives of dreams with nonsexual symbolism are his own dreams, and his examples of dreams containing sexual symbolism are described as other people’s dreams.

CONCEPT CHECK

2. A popular paperback purports to tell you what your dreams mean. The author says that every element of a dream has a symbolic meaning—in many cases, a sexual meaning. A ballpoint pen represents a penis, for example, and walking up a flight of stairs represents sexual arousal. Would Freud agree or disagree with the author of this book? (Check your answer on page 583.)

Transference

Some clients show either exaggerated love or hatred for their therapist, which seems inappropriate under the circumstances. Psychoanalysts call this reaction **transference**; they mean that clients are transferring onto the therapist what they actually feel toward their father or mother or another important figure. Transference often provides clues to the client's feelings about those people.

Psychoanalysts are fairly active in their **interpretation** of what the client says—that is, they explain the underlying meaning—and may even argue with the client about interpretations. They may regard the client's disagreement as resistance, continued repression that interferes with therapy. Resistance can take many forms; for example, a client who has begun to touch on an extremely anxiety-provoking topic may turn the conversation to something trivial or may simply “forget” to come to the next session.

Psychoanalysts today remain largely loyal to Freud's approach, although they modify it in many ways (Karon & Widener, 1995). The goal is still to bring about a major reorganization of the personality, changing a person from the inside out, by helping people understand the hidden reasons behind their actions.

Behavior Therapy

Behavior therapists assume that human behavior is learned and that it can be unlearned. They identify the behavior that needs to be changed, such as a phobia or an addiction or a nervous twitch, and then set about changing it through reinforcement, punishment, and other principles of learning. They may try to understand the causes of the behavior, as a first step toward changing it, but unlike psychoanalysts, they are more interested in directly changing behaviors than in understanding their hidden meanings.

Behavior therapy begins with clear, well-defined behavioral goals, such as eliminating test anxiety or getting the client to quit smoking, and then attempts to achieve those goals through learning. Setting clear goals enables the therapist to judge whether the therapy is succeeding. As in the examples of operant conditioning that we encountered in Chapter 6, the psychologist uses a shaping procedure—that is, starts by reinforcing simple behaviors and then progresses to more complex ones. If the client shows no improvement after a few sessions, the therapist tries a different procedure. We shall consider three examples of behavior therapy here and more in Chapter 16.

Behavior Therapy for Anorexia Nervosa

A behavior therapist treats anorexia nervosa (Chapter 11) by providing reinforcement for eating or gaining weight. For example, one woman with severe anorexia nervosa was isolated from her family and placed in a small, barren hospital

room (Bachrach, Erwin, & Mohr, 1965). She was told that she could not leave the room and could see no one except the nurse who came at mealtimes. She could obtain privileges—such as having a television or reading material, the right to leave the room, or the right to have visitors—only as a reward for gaining weight. This method may seem heartless, but life-threatening cases like this one demand drastic measures. The woman gradually gained weight and was released from the hospital when she reached 77 pounds (35 kilograms). After leaving, she lost some of the weight she had gained but not enough to endanger her life.

Behavior therapy can be combined with other therapeutic methods. In cases of anorexia nervosa, this therapy is most effective when it is combined with family counseling designed to alter the family interactions that may have led to the problem (Russell, Szmulker, Dare, & Eisler, 1987).

Behavior Therapy for Bed-Wetting

Some children continue to wet the bed long after the usual age of toilet training. Most of them outgrow the problem, but it occasionally lingers on to age 5, 10, or even into the teens.

We now know that many bed wetters have small bladders and thus have difficulty getting through the night without urinating. We also know that many are unusually deep sleepers who do not wake up when they wet the bed (Stegat, 1975).



Behavior therapists treat anorexia nervosa patients by providing privileges as reinforcements for weight gain. A life-threatening disorder requires drastic interventions.

The most effective procedure uses classical conditioning to train the child to wake up at night and go to the bathroom when the bladder is full (Houts, Berman, & Abramson, 1994). The simplest procedure uses a small battery-powered device that fits into the child's underwear at night. During the night, when the child urinates, the device detects the moisture and produces a pulsing vibration that awakens the child. (Alternative devices work on the same principle but produce loud noises.)

The vibration acts as an unconditioned stimulus (UCS) that evokes the unconditioned response (UCR) of waking up. In this instance, the body itself generates the conditioned stimulus (CS): the sensation produced by a full bladder (Figure 15.6). Whenever that sensation is present, it serves as a signal that the vibration is imminent. After a few pairings (or more), the sensation of a full bladder is enough to wake the child.

Actually, the situation is a little more complicated, because the child is positively reinforced with praise for waking up to use the toilet. Thus, the process includes both classical and operant conditioning. Training by use of an alarm or vibration eliminates bed-wetting in most children, though not all, and is sometimes effective after as little as one night.

Aversion Therapy

Although behavior therapists rely mostly on positive reinforcement, they occasionally use punishment to try to teach clients an aversion (dislike) to a stimulus. For example, they might ask someone who is trying to quit smoking to smoke twice as many cigarettes as usual for a few days, to inhale rapidly (one puff every 6 seconds), or to smoke nonstop in a small, airtight room with overflowing ashtrays, until there is little oxygen left to breathe. The goal is to teach the client

A small device called a “Potty Pager” fits into a child's underwear and produces a vibration when it becomes moist. It thereby awakens the child, who then learns to awaken when the bladder is full. (Photo courtesy of Ideas for Living, Inc., Boulder, CO.)

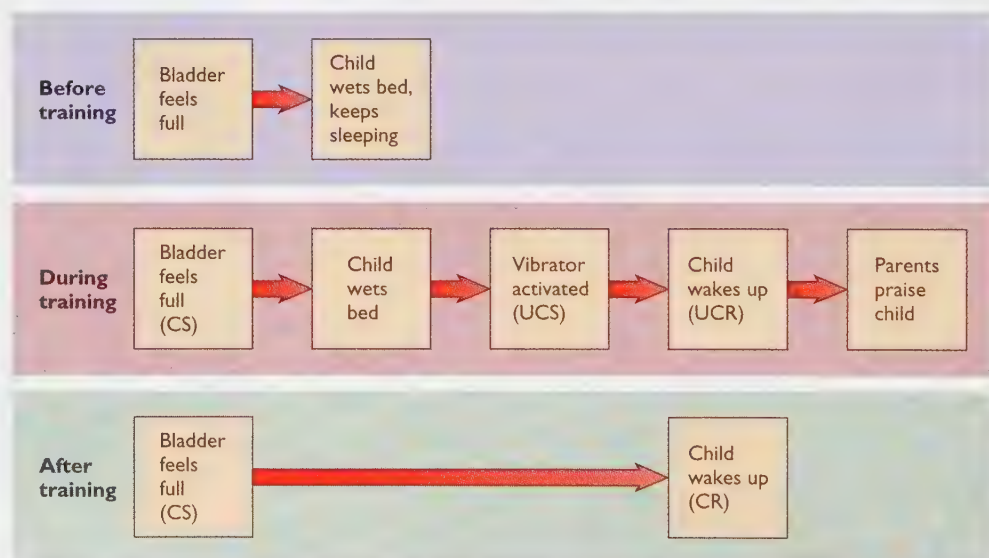


an aversion to smoking. At the end of this treatment, most people stop smoking at least temporarily, although they are likely to start up again within a year (Poole, Sanson-Fisher, & German, 1981). It is apparently difficult to undo years of enjoyable smoking with a few unpleasant sessions of aversion therapy.

Therapies That Focus on Thoughts and Beliefs

Someone says to you, “Look how messy your room is! Don't you ever clean it?” How do you react? You might say, “Big deal. Maybe I'll clean it tomorrow.” Or you might feel worried, angry, or even depressed. If you get upset, it may not be merely because you were criticized but because you want everyone to believe that you are scrupulously clean and tidy. Some therapists focus on the thoughts and beliefs that underlie people's emotional reactions. Unlike psychoanalysts, these therapists are more concerned about what their clients are thinking right now than about the early experiences that led to these thoughts.

FIGURE 15.6 A child can be trained not to wet the bed by the use of classical-conditioning techniques. At first, the sensation of a full bladder (the CS) produces no response, and the child wets the bed. This causes a vibration or other alarm (the UCS), and the child wakes up (the UCR). By associating the sensation of a full bladder with a vibration, the child soon begins waking up to the sensation of a full bladder alone, and will not wet the bed.



Cognitive and Rational-Emotive Therapies

Cognitive therapy seeks to improve people's psychological well-being by changing their cognitions—their thoughts and beliefs (Beck, 1976; Hollon & Beck, 1979). For example, someone who has undergone an extremely stressful experience, such as being raped or witnessing a fatal accident, may harbor irrational and harmful beliefs such as “it was my fault” or “it will happen to me again” (Marks, Lovell, Noshirvani, Livanou, & Thrasher, 1998). A cognitive therapist tries to identify these thoughts and encourage the client to explore the evidence behind them. Generally, the client discovers that these beliefs are unjustified. Eventually, the client learns to identify and challenge his or her own disruptive thoughts. (We shall discuss cognitive therapy for depression in Chapter 16.)

A related approach, rational-emotive therapy is a treatment based on the assumption that people's emotions depend on their “internal sentences,” such as “I can't be happy unless I succeed at all times” (Dryden & DiGiuseppe, 1990). This therapy is called “rational-emotive” because it assumes that thoughts (rationality) lead to emotions. The problem therefore is not the unpleasant emotions themselves, but the irrational thoughts that lead to them.

Rational-emotive therapists (Ellis, 1987) believe that abnormal behavior often results from such irrational beliefs as these:

- I must perform certain tasks successfully.
- I must perform well at all times.
- I must have the approval of certain people at all times.
- Others must treat me fairly and with consideration.
- I must live under easy, gratifying conditions.

The word *must* makes these beliefs irrational. Rational-emotive therapists try to identify irrational beliefs (which people may never have verbalized) and then contradict them. They urge clients to substitute other, more realistic “internal sentences.” Rational-emotive therapists intervene directly to instruct and persuade. Here is an excerpt from a rational-emotive therapy session with a 25-year-old physicist:

Client: The whole trouble is that I am really a phony. I am living under false pretenses. And the longer it goes on, the more people praise me and make a fuss over my accomplishments, the worse I feel.

Therapist: What do you mean you are a phony? I thought that you told me, during our last session, that your work has been examined at another laboratory and that some of the people there think your ideas are of revolutionary importance.

Client: But I have wasted so much time. I could be doing very much better. . . . Remember that book I told you I was writing . . . it's been three weeks now since I've

spent any time on it. And this is simple stuff that I should be able to do with my left hand while I am writing a technical paper with my right. I have heard Bob Oppenheimer reel off stuff extemporaneously to a bunch of newspaper reporters that is twice as good as what I am mightily laboring on in this damned book!

Therapist: Perhaps so. And perhaps you're not quite as good—yet—as Oppenheimer or a few other outstanding people in your field. But the real point, it seems to me, is that . . . here you are, at just twenty-five, with a Ph.D. in a most difficult field, with an excellent job, much good work in process, and what well may be a fine professional paper and a good popular book also in progress. And just because you're not another Oppenheimer or Einstein quite yet, you're savagely berating yourself.

Client: Well, shouldn't I be doing much better than I am?

Therapist: No, why the devil should you? As far as I can see, you are not doing badly at all. But your major difficulty—the main cause of your present unhappiness—is your utterly perfectionistic criteria for judging your performance. (Ellis & Harper, 1961, pp. 99–100)

CONCEPT CHECK

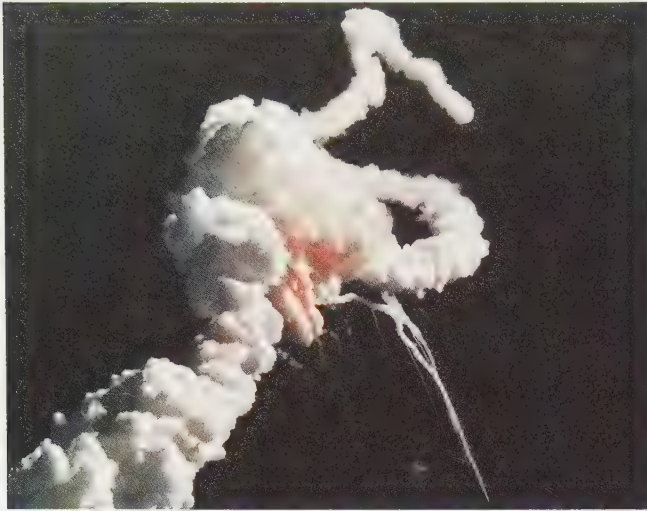
3. How does the concept behind rational-emotive therapy compare to the James-Lange theory of emotions, from Chapter 12? (Check your answer on page 583.)

Cognitive-Behavior Therapy

Many therapists combine important features of both behavior therapy and cognitive therapy to form cognitive-behavior therapy. Cognitive-behavior therapists set explicit goals for changing people's behavior, but in contrast to most behavior therapists, they place more emphasis on changing people's interpretation of their situation. For example, most of us get very upset if we see a video news report showing a fatal automobile accident; we would be much less upset if someone told us that the film was just a special-effects simulation (Meichenbaum, 1995). Similarly, cognitive-behavior therapists try to help clients distinguish between serious, real problems and imagined or distorted problems. They help clients change their interpretations of past events, current concerns, and future possibilities. Cognitive-behavior therapy has become one of the most widespread forms of therapy in the United States.

Humanistic Therapy

As we saw in Chapter 13, humanistic psychologists believe that people can decide consciously and deliberately what kind of person to be and that people naturally strive to



The crash of the *Challenger* (left) was deeply upsetting to a great many people. Tragedies in fictional films, like the one in *Con Air* shown at right, are far less disturbing. The impact of any event depends not just on the event itself, but also on how we interpret it.

achieve their full potential. However, people sometimes learn to dislike themselves because others criticize and reject them. They become distressed by the **incongruence (mismatch) between their self-concept and their ideal self**. Humanistic therapists hold that, once people are freed from the inhibiting influences of a rejecting society, they can solve their own problems.

The best-known version of humanistic therapy is **person-centered therapy** pioneered by Carl Rogers. It is also known as **nondirective** or **client-centered therapy**. The therapist listens to the client sympathetically, with total acceptance and unconditional positive regard, like the love of a parent for a child. Most of the time, the therapist restates what the client has said, to clarify it, thus conveying the message, "I'm trying to understand your experience from your point of view." The therapist strives to be genuine, empathic, and caring, rarely offering any interpretation or advice. Here is an example (shortened from Rogers, 1951, pp. 46–47):

Client: I've never said this before to anyone. This is a terrible thing to say, but if I could just find some glorious cause that I could give my life for I would be happy. I guess maybe I haven't the guts—or the strength—to kill myself—and I just don't want to live.

Counselor: At the present time things look so black to you that you can't see much point in living.

Client: Yes. I wish people hated me, because then I could turn away from them and could blame them. But no, it is all in my hands. I either fight whatever it is that holds me in this terrible conflict, or retreat clear back to the security of my dream world where I could do things, have clever friends, be a pretty wonderful sort of person.

Counselor: It's really a tough struggle, digging into this like you are, and at times the shelter of your dream world looks more attractive and comfortable.

Client: My dream world or suicide.

Counselor: Your dream world or something more permanent than dreams.

Client: Yes. (A long pause. Complete change of voice.) So I don't see why I should waste your time. I'm not worth it. What do you think?

Counselor: It's up to you, Gil. It isn't wasting my time. I'd be glad to see you, whenever you come, but it's how you feel about it. If you want to come twice a week, once a week, it's up to you.

Client: You're not going to suggest that I come in oftener? You're not alarmed and think I ought to come in every day until I get out of this?

Counselor: I believe you are able to make your own decision. I'll see you whenever you want to come.

Client: I don't believe you are alarmed about . . . I see. I may be afraid of myself, but you aren't afraid for me.

The therapist provides an atmosphere in which the client can freely explore feelings of guilt, anxiety, and hostility. By accepting the client's feelings, the therapist conveys the message, "You can make your own decisions. Now that you are more aware of certain problems, you can deal with them constructively yourself."

CONCEPT CHECK

4. Answer the following questions with reference to psychoanalysis, cognitive or rational-emotive therapy, humanistic therapy, and behavior therapy.
 - a. With which type of therapy is the therapist least likely to offer advice and interpretations of behavior?
 - b. Which type focuses more on changing what people do than on exploring what they think?
 - c. Which two types of therapy try to change what people think? (Check your answers on page 583.)

Family Systems Therapy

In family systems therapy, the guiding assumption is that a troubled individual is probably part of a troubled family. In some cases, the person who first seeks treatment is the least disturbed member of the family; in any case, therapists are usually more effective if they can deal with a person's problems in the context of other family members. Marriage counseling is a special kind of family system therapy, in which the therapist deals with a married couple.

Family systems therapy is not exactly an alternative to other forms of therapy; a family therapist can use psychoanalysis, behavior therapy, cognitive therapy, or any other technique. What distinguishes family therapists is that they prefer to talk with two or more members of a family together; even when they do talk with just one member of a family, they focus on how that individual fits into the family. Their assumption is that many problems arise from poor communication or unreasonable expectations within the family. Even when the problem is primarily that of an individual (such as alcoholism), other family members have learned to treat the disordered person in certain ways; solving the problem requires changing the family environment as well as changing the individual's behavior (Clarkin & Carpenter, 1995; Rohrbaugh, Shoham, Spungen, & Steinglass, 1995).

For example, one young man who had been caught stealing a car was taken to a psychologist. The psychologist, a family therapist, asked to talk with the parents as well. As it turned out, the father had been a heavy drinker until his boss pressured him to quit drinking and join Alcoholics Anonymous. Until that time, the mother had made most of the family decisions in close consultation with her son, who had become almost a substitute husband. When the father quit drinking, he began to assume more authority over the



Most people with psychological problems are members of troubled families. A family systems therapist focuses on how each individual's problems fit into the family's problems.

family, and his son came to resent him. The mother felt less needed and grew depressed. Each member of the family had problems that they could not resolve separately. The therapist worked to help the father improve his relationship with both his son and his wife and to help all three find satisfying roles within the family (Foley, 1984).

General Trends in Psychotherapy

Hundreds of other types of therapy are available, including some that are quite different from the five listed here. About half of all U.S. psychotherapists, however, profess no strong allegiance to any single form of therapy. Instead, they practice **eclectic therapy**, meaning that they use a combination of methods and approaches. An eclectic therapist might use behavior therapy with one client and rational-emotive therapy with another or else start with one therapy and then shift to another when the first seems ineffective. The therapist might also borrow insights from several approaches. Table 15.3 contrasts five major types of psychotherapy.

The early practitioners of psychotherapy insisted on seeing each client one at a time, frequently, and on a long-term basis. Over the years, new methods have arisen for making psychotherapy less costly, less time-consuming, and therefore available to more people. We shall now consider the alternatives of brief therapy, group therapy, and self-help groups.

Brief Therapy

Psychotherapy can require a major commitment of time and money. An individual psychotherapy session generally lasts about 50 minutes; depending on the therapist and the nature of the client's problems, the sessions are generally scheduled once per week but can be as often as every day. If the therapy begins with an open-ended plan to continue "as long as necessary," it can easily drag on for years. Indeed, many therapists, especially psychoanalysts, have assumed that frequent sessions conducted over a long period of time are essential for a successful outcome, and they regarded any client who quit after a few sessions as a dropout, a failure.

Eventually, therapists realized that some of the apparent failures were really "premature successes" who did not need further treatment. In fact, about half of all people who enter psychotherapy show significant improvement within 8 sessions, and three-fourths show improvement within 26 sessions (Howard, Kopta, Krause, & Orlinsky, 1986).

As a result, many therapists began to plan limits on the duration of therapy. At the start of **brief therapy**, or time-limited therapy, the therapist and the client reach an agreement about what they can expect from each other and how long the treatment will last—such as once per week for 2 months (Koss & Butcher, 1986). As the deadline

TABLE 15.3 Comparison of Five Types of Psychotherapy

TYPE OF PSYCHOTHERAPY	THEORY OF WHAT CAUSES PSYCHOLOGICAL DISORDERS	GOAL OF TREATMENT	THERAPEUTIC METHODS	ROLE OF THERAPIST
<i>Psychoanalysis</i>	Unconscious thoughts and motivations	To bring unconscious thoughts to consciousness; to achieve insight	Free association, dream analysis, and other methods of probing the unconscious mind	To interpret associations
<i>Cognitive therapies</i>	Irrational beliefs and unrealistic goals	To establish realistic goals, expectations, and interpretations of a situation	Dialogue with the therapist	To help the client reexamine assumptions
<i>Humanistic (person-centered) therapy</i>	Reactions to a rejecting society; incongruence between self-concept and ideal self	To enable client to make personal decisions; to promote self-acceptance	Client-centered interviews	To focus the client's attention; to provide unconditional positive regard
<i>Behavior therapy</i>	Learned inappropriate, maladaptive behaviors	To change behaviors	Positive reinforcement and other learning techniques	To develop, direct, and evaluate the behavior therapy program
<i>Family systems therapy</i>	Distorted communication and confused roles within a family	To improve the life of each individual by improving functioning of the family	Counseling sessions with whole family or with individual talking about life in the family	To promote better family communication and understanding

approaches, both the therapist and the client are strongly motivated to bring the therapy to a successful conclusion. (How hard would you work on a term paper if you had no deadline to meet? How would a professor conduct the class if a course were scheduled to continue “as long as necessary”? Without deadlines, few of us would apply ourselves as diligently as possible.)

Moreover, with a deadline that's agreed-upon in advance, clients do not feel deserted or rejected when the therapy ends. They may return for an occasional extra session months later, but for a time they must get along without help. Any client who fails to make progress by the deadline should consider going to a different therapist. Many clients, probably most, prefer brief therapy to an indefinitely long commitment, and most clients with mild problems respond well to brief therapy.

Unfortunately, however, recent economic trends have created pressure for therapy to be extremely brief, sometimes too brief. People who receive health care through health maintenance organizations (HMOs) pay a fixed amount per month and then receive whatever medical care they need from the staff of the organization. Most HMOs offer brief therapy or “crisis intervention” as part of their services. In other words, anyone who belongs to such an HMO can get a few psychotherapy sessions (typically 3–20, depending on the HMO), without any extra payment. The advantage of this system is that many people who could not otherwise afford psychotherapy can get help during a time of distress (Hoyt & Austad, 1992). The disadvantage is that the HMO saves money by minimizing ser-

vices and therefore authorizes only brief therapy even for people with more extensive problems. Any client who needs more than the allotted amount of treatment is either given a prescription for drugs instead of psychotherapy or encouraged to see a therapist outside the HMO at the client's own expense (Karon, 1995).

Economically speaking, it is impossible to maximize the quality of care and minimize the cost at the same time. The goal of HMOs, to provide good yet inexpensive therapy, is understandable; however, some HMOs provide simply too little. Working out a cost-effective solution will be a serious challenge.

Group Therapy

The pioneers of psychotherapy dealt with their clients on a one-on-one basis. Individual psychotherapy has advantages—most of all, privacy. But for many purposes, it is helpful to treat clients in groups. Group therapy is therapy that is administered to a group of people all at once. It first became popular as a method of providing help to people who could not afford to pay for individual sessions. (Spreading the costs of a session among 5–10 group members reduces the cost for each.) Eventually, therapists found that group therapy has other advantages as well. In particular, many clients seek help because of failed relationships or because they have trouble dealing with other people. A group therapy session enables them to examine how they relate to others and to practice better social skills (Ballinger & Yalom, 1995).



Group therapy can be less expensive than individual therapy. Moreover, participants learn from one another and explore their ways of relating to other people.

Self-Help Groups

Self-help groups, such as Alcoholics Anonymous, operate much like group therapy sessions, except that they do not include a therapist. Each participant both gives and receives help. People who have experienced a particular problem themselves can offer special insights to others with the same problem. They are especially well prepared to deal with someone who says, “You just don’t understand.” They reply, “Oh, yes, we do!” Self-help groups have another advantage: The members are available to help one another whenever someone needs help—often or seldom, without an appointment, without charge.

Some self-help groups are composed of current or former mental patients. The members feel a need to talk to others who have gone through a similar experience, either in addition to or instead of treatment by a therapist. The Mental Patients’ Association in Canada was organized by former patients who were frustrated and angry at the treatment they had received (or failed to receive), especially in mental hospitals (Chamberlin, 1978). Similar organizations in the United States and Europe enable former patients to share experiences with one another, provide support, and work together to defend the rights and welfare of mental patients.

Evaluating the Effectiveness of Psychotherapy

For the purpose of healing what ails your psyche, you can talk to someone who will be very curious about your childhood toilet training, someone who will compare your dreams to ancient myths and folktales, someone who will listen sympathetically and wait for you to solve your own problems, or someone who will use learning principles to change your habits. Hundreds of forms of therapy are available, ranging from traditional, mainstream approaches to

some odd and untested procedures. You can, if you wish, crawl naked into a hot tub with your therapist to reenact the moment of birth. However, if you want your insurance company to pay for your treatment, the company will question whether the treatment will be effective enough to be worth the cost.

How can we measure the effectiveness of a treatment? You might imagine that it would be simple to determine how much people improve in their psychological well-being from the start to the end of treatment. However, when would you guess that people usually enter psychotherapy—when they are feeling better than usual or when they are feeling worse? Naturally, they seek therapy when they are feeling worse, perhaps even the worst they have ever felt. From that starting point, they have almost nowhere to go but up. That is, an apparent improvement in someone’s condition might be due to natural fluctuations in psychological well-being instead of the results of therapy.

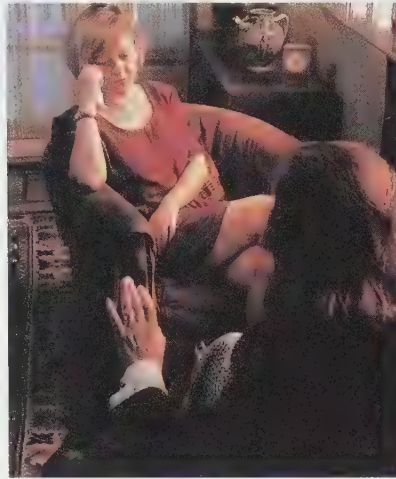
Hans Eysenck (1952) called attention to this issue by pointing out that about 65% of the people who never receive therapy for their psychological problems nevertheless improve in a year or two. Improvement without therapy is called spontaneous remission. So, to measure the effectiveness of psychotherapy, an investigator must compare the improvement of psychotherapy clients to the spontaneous remission rate.

WHAT IS THE EVIDENCE?

How Effective is Psychotherapy?

In other chapters in this book, the “What’s the evidence?” section has highlighted a particular study. Here, I want to describe a general research approach. Hundreds of research studies similar to this have been conducted, although each of them varies in its details (Kazdin, 1995).

Because everyone's moods and effectiveness fluctuate over time, an apparent improvement between the start and the end of therapy is hard to interpret. How much of the improvement depended on the therapy and how much would have occurred even without therapy? Adequate research on this issue requires a control group.



Hypothesis Psychologically troubled people who receive psychotherapy will show greater improvements in their condition than similar people who do not receive therapy.

Method For the results to be meaningful, participants must be assigned at random to the therapy and nontherapy groups. (Comparing people who sought therapy to those who did not seek it would be unfair, because the two groups might differ in the severity of their problems or their motivation for overcoming them.) In the best studies, people who contact a clinic about receiving therapy are all given a preliminary examination and are then randomly assigned to receive therapy at once or to be placed on a “waiting list” for therapy at a later time. A few months later, the investigators compare the amount of improvement shown by the therapy group and the waiting-list group.

How should the investigators measure the amount of improvement? Researchers cannot rely on the judgments of the therapists, who, after all, want to demonstrate the effectiveness of their procedures. For similar reasons, they cannot ask the clients for an unbiased opinion about how much they have improved. Therefore, the researchers may ask a “blind” observer (see Chapter 2) to evaluate each client, without knowing who has received therapy and who has been on the waiting list. Or they may ask each person to take a standardized personality test, such as the MMPI. Unfortunately, the personality tests are far from perfect, and a single evaluation by a blind observer uncovers only a limited amount of information. In short, this research method should be able to detect major changes in clients' well-being, but it may overlook smaller changes.

Many experiments on psychotherapy simply compare a group that received therapy to a control group that was on the waiting list. Other experiments, however, have compared groups receiving different kinds of therapy or different frequencies of therapy.

Results Here, we are not interested in the results of any single study. Most experiments have included only a modest number of people, such as 10 or 20 receiving therapy and a

similar number on the waiting list. To draw a conclusion, we need to pool the results from a great many similar experiments. Psychologists use a method called meta-analysis, taking the results of many experiments, weighting each one in proportion to the number of participants, and determining the overall average effect. According to one meta-analysis that pooled the results of 475 experiments, the average person in therapy shows greater improvement than 80% of similarly troubled people who are not receiving therapy (Smith, Glass, & Miller, 1980). Figure 15.7 illustrates this effect.

Interpretation One could easily complain that investigators have invested a great deal of effort for rather little pay-off. After 475 experiments, we can confidently say that getting therapy is usually better than not getting therapy. This conclusion is like saying that, if you are ill, some medicine is better than no medicine. However, even if this conclusion seems unimpressive, it does pave the way for further, more detailed studies about which kinds of therapy are best, how therapy produces its benefits, and so forth.



CONCEPT CHECK

- Although well-designed experiments on psychotherapy use a blind observer to rate clients' mental health, double-blind studies are virtually impossible. Why? (Check your answer on page 583.)

Comparisons of Different Therapies and Therapists

Next, of course, we want to know which kinds of therapy are best and for which disorders these are most effective. The practical problem for researchers is that therapists have identified hundreds of psychological disorders and have developed hundreds of types of therapy. To test each form of therapy on each possible disorder would require tens of

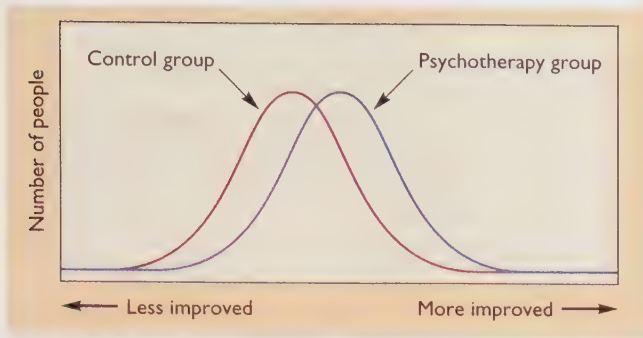


FIGURE 15.7 According to one review of 475 studies, the average person receiving psychotherapy shows more improvement than do 80% of similar, randomly assigned people who are not in therapy. Note that both groups show substantial variation; some untreated people progress better than some treated people. This comparison lumps together all kinds of therapy and all kinds of troubled people. (From Smith, Glass, & Miller, 1980.)

thousands of experiments, each using as many participants as possible. Furthermore, many clients have a mixture of several problems, not just one, and many therapists use an eclectic, trial-and-error approach instead of a single, well-defined therapy (Goldfried & Wolfe, 1996). Under these circumstances, it would be highly impractical to test every therapy for every disorder. However, the results of the studies that have been done support a stunningly simple conclusion: For the disorders that have been widely tested, all the common, mainstream forms of therapy are approximately equal in effectiveness (Lipsey & Wilson, 1993; Stiles, Shapiro, & Elliott, 1986; Wampold et al., 1997). That finding is surprising, given that psychoanalysis, behavior therapy, person-centered therapy, and the others differ so sharply in their assumptions, methods, and goals. If all treatments succeed almost equally well, they must resemble one another more than we had thought.

Research has also compared the effectiveness of thera-

pists with different kinds of training. The U.S. magazine *Consumer Reports* (1995) surveyed its readers about their mental health and their contact with psychotherapy. Of the thousands who said they had sought help for a mental health problem within the previous 3 years, most said they were satisfied with the treatment and thought it had helped them. For measuring the effectiveness of psychotherapy, this study has some obvious problems: the lack of a random sample, the lack of a control group, and reliance on clients to evaluate their own improvement (Jacobson & Christensen, 1996). Nevertheless, the results indicated that people reported about equal satisfaction and benefits from talking with a psychiatrist, a psychologist, or a social worker (Seligman, 1995). They reported somewhat less satisfaction from consulting a marriage counselor, but presumably the people who consulted marriage counselors had different, and perhaps especially difficult, problems. (See Figure 15.8.)

The same general pattern emerged in a variety of other studies that have used more traditional experimental methods, though with smaller samples of people: Therapy generally helps, but the type of therapist is not critical. In fact, studies have even compared the results for clients who were randomly assigned to talk with either an experienced therapist or a caring individual who had no formal training in psychology. Most clients gained just about as much from consulting an inexperienced as an experienced therapist. In short, we have no concrete evidence that the extensive experience or training of a therapist increases the effectiveness of psychotherapy (Christensen & Jacobson, 1994; Dawes, 1994).

Is the conclusion, then, that if you are psychologically troubled, you may as well talk to your next-door neighbor instead of a psychotherapist? No, for several reasons:

- The study just described dealt with clients who had mild problems, and we cannot assume that the same results would hold true for other clients.

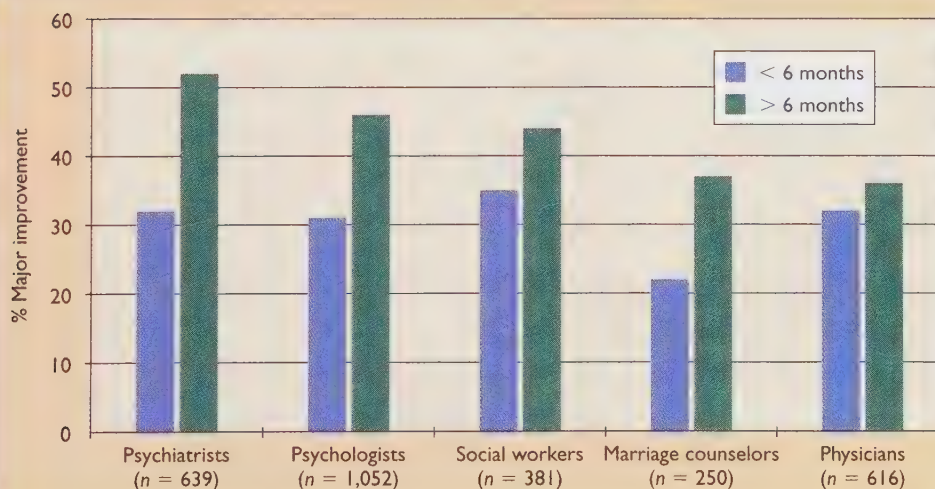


FIGURE 15.8 The percentage of *Consumer Reports* respondents who said they experienced much improvement in the problem that led them to treatment. For those who visited therapists for less than 6 months, the reported improvements were the same after visits to psychiatrists, psychologists, social workers, or family physicians. Marriage counselors, who deal with different kinds of problems, had somewhat lower ratings. (From Seligman, 1995.)



The research indicates that talking with an untrained but caring person is about as helpful as talking with an experienced therapist, at least for people with mild problems. Seeing a therapist does have advantages, however, including guaranteed privacy.

- Few of us have a friend or next-door neighbor with enough patience to listen to hours of personal ramblings and occasional outbursts of anger.
- Conversation with a professional psychotherapist is completely confidential. You cannot be sure that a friend will keep your comments secret.
- A well-trained psychotherapist can recognize symptoms of a possible brain tumor or other medical disorder and is more likely than someone else to refer you to an appropriate medical specialist.
- Clinical psychologists know enough about therapy research that they are less likely than other people to follow untested “fad” treatments (Maki & Syman, 1997).

In short, although you would probably benefit by talking to almost any sympathetic listener, there are important advantages to seeing a professional psychotherapist (Strupp, 1996).

Is More Treatment Better?

If it helps to meet with a therapist once per week for 3 months, would meeting one for 6 months help even more? Would meeting twice per week for a year help even more? How much is the right amount?

Respondents in the *Consumer Reports* (1995) study mentioned earlier were asked how much treatment they got and how much it helped them. Generally, those who received longer treatment reported that it helped them more. However, this was not an experimental study, so people were not randomly assigned to groups receiving different amounts of treatment. It may well be that the people who stayed in treatment the longest were those with the greatest problems at the start and therefore with the greatest room for improvement.

A somewhat better research design examines a single group of clients repeatedly as all progress through a given number of therapy sessions. Figure 15.9 shows the results of one such study. According to both the researchers and the clients themselves, most clients showed fairly rapid progress at first and then gradual progress that continued over 2 years (Howard, Kopta, Krause, & Orlinsky, 1986). Evidently, prolonged treatment is at least somewhat more beneficial than brief treatment, on the average.

The most elaborate study of this issue so far was a 5-year study conducted at Fort Bragg, North Carolina. A government-supported program provided free clinical services for every teenager or child needing psychological help who had a parent in the military. Each of these clients had a case manager who determined what plan of treatment was best, made sure that the client received every needed type of help, and coordinated all the service providers to make sure that each one knew what the others were doing for the client. The goal was to demonstrate that a well-planned, integrated program of treatment would be more effective than the usual forms of treatment, and that it might even be less expensive, because there would be less waste and overlap of services. Results were compared to those in a similar community that offered the usual sort of less coordinated services. The result? The integrated program at Fort Bragg provided no more benefits. It was just more expensive. (Bickman, 1996). We should not draw too strong a conclusion from this study; after all, it was not a true experiment (people were not randomly assigned to groups), and the design of the integrated program at Fort Bragg may not have been as good as what someone else might design. Still, most people had expected the integrated pro-

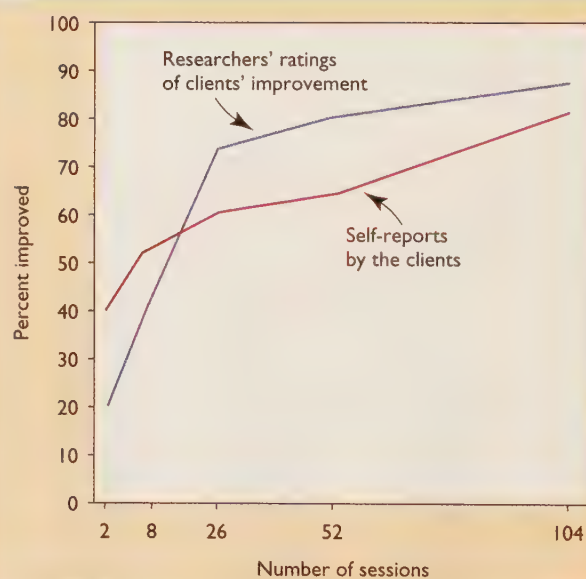


FIGURE 15.9 The relationship of the number of psychotherapy sessions and the percentage of clients who improved. (From Howard et al., 1986.)

gram to show some advantages, and the results do seem to imply that more therapy is not always better.

Similarities Among Psychotherapeutic Methods

Because all the common forms of psychotherapy have been shown to be almost equally effective, they presumably share some important features. One point in common is that all rely on the “therapeutic alliance”—a relationship between therapist and client that is characterized by acceptance, caring, respect, and attention. This relationship provides the social support that helps clients to deal with their problems and acquire social skills that they can apply to other relationships (Krupnick et al., 1994).

Moreover, in nearly all forms of therapy, clients talk openly and honestly about their beliefs and emotions, relationships with family members, and other issues that people ordinarily keep secret. They examine aspects of themselves that they usually take for granted; in so doing, they gain self-understanding. Because of the expectation of self-disclosure, psychotherapy as practiced in North America and Europe does not transplant readily into other cultures. For example, most Chinese would consider it shameful to discuss personal or family matters with a stranger (Bond, 1991).

The mere fact of entering therapy, whatever the method used, improves clients’ morale. The therapist conveys the message “you are going to get better.” Clients thus begin to think of themselves as people who can cope with their problems and overcome them. Just expecting improvement can lead to improvement.

Finally, every form of therapy requires clients to commit themselves to making some sort of change in their lifestyle. Simply by coming to the therapy session, they are reaffirming their commitment—to drink less, to feel less depressed, or to overcome their fears. They are also obliged to work on that change between sessions, so that they can come to the next session and report, “I’ve been doing a little better lately.” Improvement may depend at least as much on what clients do between sessions as on what happens in the sessions themselves.

Advice for Potential Psychotherapy Clients

At some point, you or someone close to you may be interested in seeing a psychotherapist. If so, here is some advice to consider:

- Consulting a therapist does not mean that something is wrong with you. Many people who consult a therapist have no diagnosable disorder; they merely want to talk with someone.
- If you live in the United States, you can look in the

white pages of your telephone directory for the Mental Health Association. Call them and ask for a recommendation of a therapist. You can specify how much you are willing to pay, what kind of problem you have, and even what kind of therapist you prefer.

- Other things being equal, you may do best with a therapist from your own cultural background (Sue, 1998). Therapists are trained to be sensitive to people from other backgrounds, but a communication barrier often remains nevertheless. For example, certain responses to grief or fear that are considered abnormal by many Western psychotherapists are considered normal in many non-Western cultures.
- If your religion is an important part of your outlook on life, you might prefer a therapist who sympathizes with your beliefs (Worthington, Kurusu, McCullough & Sandage, 1996). Conversely, of course, if you are not devoutly religious, you might want to avoid a therapist who wants to focus on religion.
- Be skeptical of any therapist who seems overconfident. Clinical experience does not give anyone quick access to your private thoughts.
- Even if your treatment is not labeled “brief therapy,” expect some improvement within 6–8 weeks. If you do not seem to be making progress, there may be a good reason. But at least, ask your therapist why your progress is slow; if you do not receive a convincing answer, consider seeing a different therapist.

THE MESSAGE

Finding the Best Therapy

The research indicates that various kinds of therapy are about equally effective for the average client and that psychiatrists, psychologists, and social workers are about equally effective therapists, on the average. These results do not mean, however, that any therapist is just as good as any other.

Although no one has done the research, chances are that one could also find that, on the average, professors using a lecture approach are neither considerably more nor less effective than those using mostly discussion. And yet you know from extensive experience that some lecturers are excellent and others are poor, and that some discussion leaders are excellent and others poor. Also, a professor who meets the needs of one student may be unsatisfactory for another. The same is true for psychotherapists. No one way of doing therapy is right for every client; the challenge is to find a good match between client and therapist.

S U M M A R Y

- ★ **Psychoanalysis.** Psychoanalysts try to uncover the unconscious reasons behind self-defeating behaviors. To bring the unconscious to consciousness, they rely on free association, dream analysis, and transference. (page 569)

- ★ **Behavior therapy.** Behavior therapists set specific goals for changing a client's behavior and use a variety of learning techniques to help a client achieve those goals. (page 571)
- ★ **Cognitive therapies.** Cognitive therapists try to get clients to give up their irrational beliefs and unrealistic goals and to replace defeatist thinking with more favorable views of themselves and the world. Many therapists combine features of behavior therapy and cognitive therapy, attempting to change people's behaviors by altering how they interpret the situation. (page 573)
- ★ **Humanistic therapy.** Humanistic therapists, including person-centered therapists, assume that, if people accept themselves as they are, they can solve their own problems. Person-centered therapists listen with unconditional positive regard but seldom offer interpretations or advice. (page 573)
- ★ **Family systems therapy.** In many cases, an individual's problem is part of an overall disorder of family communications and expectations. Family systems therapists try to work with as many family members as possible. (page 575)
- ★ **Eclectic therapy.** About half of all psychotherapists today call themselves "eclectic"—that is, they use a combination of methods, depending on the circumstances. (page 575)
- ★ **Brief therapy.** Many therapists set a time limit for the treatment, usually ranging from 2–6 months. Brief therapy is usually about as successful as long-term therapy, if the goals are limited. (page 575)
- ★ **Group therapies and self-help groups.** Psychotherapy is sometimes provided to people in groups, often composed of people with similar problems. Self-help groups provide sessions similar to group therapy but without a therapist. (page 576)
- ★ **Effectiveness of psychotherapy.** The average troubled person in therapy improves more than at least 80% of people who are not in therapy. On the average, each of the common forms of therapy provides approximately equal benefits for the disorders that have been extensively tested. Psychiatrists, psychologists, and social workers also provide approximately equal benefits, on the average. (page 577)
- ★ **Similarities among therapies.** A wide variety of therapies share certain features: All rely on a caring relationship between therapist and client. All promote self-understanding. All improve clients' morale. And all require a commitment by clients to try to make changes in their lives. (page 581)

Suggestions for Further Reading

- Bongar, B., & Beutler, L. E. (1995). *Comprehensive textbook of psychotherapy*. Oxford, England: Oxford University Press. Describes the theoretical assumptions and procedures of the most common forms of psychotherapy.
- Dawes, R. M. (1994). *House of cards: Psychology and psychotherapy built on myth*. New York: Free Press. A harsh criticism of psychotherapists who rely on their intuition and experience instead of scientific evidence.
- Seligman, M. E. P. (1993). *What you can change . . . and what you can't*. New York: Fawcett Columbine. Description of both the possibilities and the limitations of psychotherapy.

Terms

- psychotherapy** the treatment of psychological disorders by methods that include a personal relationship between a trained therapist and a client (page 569)
- psychodynamic therapies** treatments that attempt to uncover people's underlying drives and motivations (page 569)
- psychoanalysis** a method of psychotherapy developed by Sigmund Freud, based on identifying unconscious thoughts and emotions and bringing them to consciousness (page 569)
- catharsis** the release of pent-up emotions associated with unconscious thoughts and memories (page 569)
- free association** a procedure where a client lies on a couch, starts thinking about a particular symptom or problem and then reports everything that comes to mind (page 570)
- manifest content** the content that appears on the surface of a dream (page 570)
- latent content** the hidden content of a dream that is represented only symbolically (page 570)
- transference** the extension of a client's feelings toward a parent or other important figure onto the therapist (page 571)
- interpretation** a therapist's explanation of the underlying meaning of what a client says (page 571)
- resistance** according to psychoanalysts, continued repression that interferes with therapy (page 571)
- behavior therapy** treatment that begins with clear, well-defined behavioral goals, such as eliminating test anxiety, and then attempts to achieve those goals through learning (page 571)
- cognitive therapy** treatment that seeks to improve people's psychological well-being by changing their cognitions (page 573)
- rational-emotive therapy** treatment based on the assumption that people's emotions depend on their "internal sentences," such as "I can't be happy unless I succeed at all times" (page 573)
- cognitive-behavior therapy** treatment that combines important features of both behavior therapy and cognitive therapy, attempting to change people's behavior by changing their interpretation of their situation (page 573)
- incongruence** a mismatch between someone's self-concept and ideal self (page 574)
- person-centered therapy** (also known as nondirective or client-centered therapy) a procedure in which a therapist listens to the client sympathetically, provides unconditional positive regard, and offers little interpretation or advice (page 574)
- family systems therapy** treatment based on the assumption that a troubled individual is probably part of a troubled family (page 575)
- eclectic therapy** treatment that uses a combination of methods and approaches (page 575)
- brief therapy** (or time-limited therapy) treatment that begins with an agreement about what the therapist and the client can expect from each other and how long the treatment will last (page 575)
- group therapy** treatment administered to a group of people all at once (page 576)
- spontaneous remission** improvement of a psychological condition without therapy (page 577)
- meta-analysis** a method of taking the results of many experiments, weighting each one in proportion to the number of participants, and determining the overall average effect (page 578)

Answers to Concept Checks

2. Freud would disagree with the premise of this paperback. Freud believed that the symbolism of dream elements differed from one person to another. (page 570)
3. Rational-emotive therapy assumes that many thoughts lead to emotions. This assumption is the reverse of the James-Lange theory, which argues that emotion-related changes in the body give rise to thoughts. (page 573)
4. **a.** Humanistic therapy; **b.** behavior therapy; **c.** psychoanalysis and cognitive therapy. (page 574)
5. A double-blind design requires that neither the subjects nor the observers know which subjects received the experimental treatment and which ones were in the control group. It is not possible to prevent subjects from knowing whether they have received psychotherapy. (page 578)

Web Resources

The Efficacy of Psychotherapy

www.apa.org/practice/peff.html

Psychotherapy

www.psych.org/public_info/psythera.html

The American Psychological Association provides information about the efficacy of psychotherapy. The American Psychiatric Association describes the general procedures involved in psychotherapy.

Health Center: Therapy

www.health-center.com/english/brain/therapy/

This site includes definitions of therapy, descriptions of several major kinds of therapy, and suggestions on how to go about seeing a therapist.

Social and Legal Issues in the Treatment of People with Disorders

What should our society do
about psychological disorders?

A group of nearsighted people, lost in the woods, were trying to find their way home. One of the few who wore glasses said, "I think I know the way. Follow me." The others burst into laughter. "That's ridiculous," said one. "How could anybody who needs glasses be our leader?"

In 1972, the Democratic party nominated Senator Thomas Eagleton for vice president of the United States. Shortly after his nomination, he revealed that he had once received psychiatric treatment for depression. He was ridiculed mercilessly: "How could anybody who needed a psychiatrist be our leader?"

A great many people suffer from a psychological disorder at some point during their life. Unfortunately, many people in our society consider it shameful to seek help for a psychological disorder. They struggle along on their own, like a nearsighted person who refuses to wear glasses, rather than admit that they need help.

As a citizen and a voter, you will have to deal with numerous issues relating to psychological disorders and therapies: Who, if anyone, should receive psychiatric treatment against their protest? Should mental patients have the right to refuse treatment? Under what circumstances, if any, should a criminal defendant be acquitted because of "insanity"? Can society as a whole take steps to prevent certain types of psychological disorders?

Mental Hospitals and Deinstitutionalization

Until the 1950s, people with severe psychological disturbances in the United States were generally confined in large mental hospitals supported by their state or county. Most of these hospitals were understaffed, poorly funded, prisonlike institutions that kept patients alive and out of other people's way but provided little if any psychiatric care (Okun, 1983).

Hospital attendants cooked the food, did the laundry, made all the decisions, but did not try to teach patients the skills they would need if they were ever to leave the institution. After all, few patients ever did. Some hospitals were better than others, but most were pretty grim places.

With the advent of antidepressant and antipsychotic drugs in the 1950s, advances in psychotherapy, and changes in the commitment laws, the number of long-term mental patients declined steadily. Today, the number of hospital beds reserved for psychiatric patients is one-fifth of what it was in 1955. A given patient may be admitted to a hospital repeatedly, but most stays are brief (Appleby et al., 1993).

Several alternatives to hospitalization are available. Many cities have homes staffed and organized to treat people with psychiatric problems. Another alternative is a community mental health center, where some patients reside but others just come for visits during the day. The idea behind these various forms of alternative care is that patients should receive appropriate supervision and treatment but should also have as much contact as possible with the "real world." Studies have consistently found that community mental health centers are less expensive than large mental hospitals and at least as effective in restoring people to independent living (Fenton, Mosher, Herrell, & Blyler, 1998).

Such research prompted a movement toward deinstitutionalization, the removal of patients from mental hospitals, to give them the least restrictive care possible. Hospital stays would be brief, except for people who were dangerous to themselves or others. Instead, people would live in or near their own communities and continue to receive outpatient care as needed.

Unfortunately, most states discharged great numbers of patients from their mental hospitals without planning adequate alternatives for their care and housing. Many of the



Mental hospitals of the 1950s were crowded, unpleasant warehouses of people who received minimal care. Many stayed for years, even for the rest of their lives.



Deinstitutionalization moved people out of mental hospitals, but many received little or no treatment after their release. Some became unemployed and homeless.

people who might once have been in a mental hospital are now in nursing homes or prisons; some are homeless; and many now receive no therapeutic attention (Smith, Schwebel, Dunn, & McIver, 1993).

Legal Issues in the Treatment of Disordered People

In a democratic society, we treasure both freedom and security. Sometimes these values are in conflict. For example, the right of a psychologically disordered person to be free may conflict with the right of other people to feel safe and secure. We shall consider several of these conflicts and the difficulties of resolving them.

Involuntary Commitment and Treatment

Suppose Charles mutters incoherently, cannot hold down a job, does not pay his bills or take care of his personal hygiene, and bothers his neighbors. His family wants to commit him to a mental hospital, and the psychiatrist wants to give him drugs. But Charles refuses both courses, claiming that his family is “out to get him.” Should he be permitted to refuse treatment?

We can argue this question either way. On the one hand, some of the most seriously disordered people fail to recognize that there is anything wrong with them. On the

other hand, some families have been known to commit annoying relatives to mental hospitals just to get them out of the way, and some psychiatrists have given drugs to people with only minor problems, thereby doing them more harm than good.

Unfortunately, it is difficult to determine who really needs help and who doesn't. Who (if anyone) should be unwillingly confined to a mental hospital? In the United States, laws vary from state to state (Weiner & Wettstein, 1993), but in all cases, a judge holds a hearing and makes the final decision. In some states, a court can commit patients to a mental hospital only if they are suffering from a mental disorder and are dangerous to themselves or others. The American Psychiatric Association has recommended changing the laws to allow commitment of nondangerous patients also, if they have a severe but treatable disorder and “lack capacity to make an informed decision concerning treatment” (Bloom & Faulkner, 1987; Hoge, Sachs, Appelbaum, Greer, & Gordon, 1988). Even where the law does require a ruling of dangerousness, however, courts sometimes declare an apparently incompetent person to be dangerous simply because of one or two threatening statements (Durham, 1996).

After people have been committed to a mental hospital, voluntarily or involuntarily, they still have the right to refuse treatment (Appelbaum, 1988). According to their psychiatrists, most of the people who refuse treatment are hostile, emotionally withdrawn, and prone to disorganized thinking (Marder et al., 1983). According to the patients themselves, they have good reason to be hostile and withdrawn; the hospital staff is trying to force them to submit to unnecessary and dangerous treatments. Figure 15.10 compares patients with schizophrenia who refused drug treatment and patients who agreed to drug treatment. Understandably, decisions about enforced treatment can be very difficult.

SOMETHING TO THINK ABOUT

Thomas Szasz (1982) proposed that psychologically “normal” people write a “psychiatric will,” specifying what treatments to give them, and what treatments to avoid, if they ever develop a severe psychological disorder. If you wrote such a will, what would you include in it? Or would you prefer to trust your judgment later, at the time of the onset of the disorder? ★

The Duty to Protect

Suppose someone tells his psychotherapist that he is planning to kill his former girlfriend. The therapist talks with the client and, by the end of the session, believes that the client has changed his mind. However, a few days later, the client actually does kill his ex-girlfriend. Should the therapist have notified the police and warned the woman of the danger? Should the woman's family be able to sue the therapist and collect damages?